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Correct Answer

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Practice Exam - 1

Question 72 of 117



A 24-year-old male presents to the emergency room with abdominal pain. He was in his usual state of health until 1 month prior when he developed a viral syndrome. His fever resolved, but he has continued to have malaise and fatigue. In the last week his malaise has gotten worse, and he has also developed abdominal pain and nausea. On physical exam he has a blood pressure of 95/60 and a pulse of 104 bpm. Jugular venous distention is present. On cardiac auscultation he has a regular rate and rhythm, a 2/6 systolic murmur and an S3. The lung fields are clear, and peripheral pulses fluctuate with each beat. His extremities are warm, and he has 1+ edema.

Which statement is true regarding the patient's physical exam

- A. Lack of pulmonary rales excludes the diagnosis of heart failure
-  B. Presence of lower extremity edema is a sensitive and specific sign of heart failure
-  C. Presence of jugular venous distention and an S3 indicate that a heart failure diagnosis is likely
- D. Fluctuations of the pulse suggest cardiac tamponade

Commentary

References

Testing Point: The Board-certified AHFTC specialist should know the most sensitive and specific physical exam findings for the diagnosis of heart failure.

Answer: C. Presence of jugular venous distention and an S3 indicate that a heart failure diagnosis is likely.

Presence of elevated jugular venous pressure and an S3 are of diagnostic utility and prognostic. An elevated jugular venous pressure is both sensitive and specific for heart failure. Furthermore, a landmark study in the New England Journal demonstrated that patients with an elevated JVP have significantly worse prognosis up to five years later.

Answer B: Though peripheral edema is specific for heart failure, it lacks sensitivity. Answer A: Rales are often absent, particularly in patients with chronic heart failure, thus are also not sensitive.

Answer D: Pulsus alternans is associated with low cardiac output, whereas pulsus paradoxus is associated with cardiac tamponade.

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